



TO SEND OUT

Conversation: Dr Yusuf Shittu

What to say to a GP who has founded three companies. Met at the APPG,
22 September 2026.

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

DATE

8 September 2026

VERSION

1.0 · Confidential draft

What to say to a GP who has founded three companies. Met at the APPG, 22 September 2026.

He is a practising GP with an NHS Clinical Entrepreneur Programme place and a design background. He has built three companies, so he has been where you are and he has failed at least once. That makes him the most useful person in the room and the easiest to talk to.

Two rules for the whole conversation. **Ask for his opinion, never for his help.** And ****let him talk more than you do.**** Founders who have been through it enjoy being asked properly.

Opening

"Are you Yusuf? Kelvin. I build maternity software — nothing live yet, no customers. You're on the Clinical Entrepreneur Programme, right? Can I ask you something I can't get a straight answer on from anyone who isn't a clinician?"

Then stop. Let him say yes. The pause matters; it makes it his conversation, not your pitch.

The question that earns the rest of the conversation

"My whole claim is that the software writes down what a woman said and when, and what happened next. So it's harder to say afterwards that she never mentioned it. Does that land with clinicians as a safety record? Or does it just sound like more admin?"

This is the crux of your product and a GP will answer it in five seconds. Whatever he says next is the most valuable thing you will hear this year. Write it down in front of him.

If he says it sounds like admin

Do not defend it. Go further in:

"That's what I was afraid of. What would make it worth the typing? Or is the honest answer that nobody adds a field to a maternity record unless someone forces them to?"

If he says it lands

"Then who feels the benefit first — the midwife, the consultant, or the trust's legal team? I've been assuming the midwife and I might have that wrong."

If he asks what it actually does

Keep it to three sentences and stop:

"Appointments, symptoms and a blood pressure monitor for the women who need watching. It's care coordination, not diagnosis. There's a prediction engine in the old version that I've pulled out, because it would make it a medical device and I can't back the claims."

That last sentence is the one that will make him take you seriously. Founders respect someone who removed a feature for the right reason far more than someone with a big roadmap.

The two things only he can tell you

Ask these second, once he is talking freely.

"How did you find your clinical safety officer? Everyone tells me I need one and nobody tells me where they come from or what they actually cost."

"Is there any route into the Clinical Entrepreneur Programme for a founder who isn't a clinician? Or does it have to be a clinical co-founder?"

Both are real questions you genuinely need answered. Neither asks him for a favour.

How to close it

"This has been more useful than the last three months of reading. Can I connect with you and send you one paragraph on what I change after tonight? No pitch, no deck. I'd just like to tell you what you changed."

Then leave. Do not extend the conversation past its natural end. Ending it yourself, early, is what makes him answer the message later.

What not to say

Do not	Why
"Would you be my clinical safety officer?"	It is a paid role carrying real legal liability. Asking in five minutes reads as naive
"Can you introduce me to..."	Not on a first meeting. Earn it with the follow-up
Anything about the raise, the valuation or the market size	He is a clinician first. It signals you care about the money before the care
"We're working with the NHS"	You are not. He would know within one question
Handing him anything printed	Nobody wants paper at a drinks reception. Send it after

The message that evening

Send it the same night, while he remembers your face. Short, specific, nothing attached.

Yusuf — Kelvin, from the Black Maternal Health group this afternoon. You said [the exact thing he said]. I've been building on the opposite assumption, so that is worth a lot to me. I'm going to [the specific change you will make]. If that turns out to be wrong too, I'd rather hear it from you early than from a trust in a year. No ask. Thank you for being straight with me.

If he replies, ask for twenty minutes on a call. Not before.

Where this could go, over months not days

Do not raise any of this on the first meeting. It is what you are playing for.

Stage	What it looks like	Realistic timing
Honest critic	He tells you what is wrong, by message	Now
Adviser	A recurring call. Unpaid at first	1–3 months
Named clinical adviser	On your website, with his permission in writing	3–6 months
Introductions	To a maternity clinician, and into the innovation networks	When he trusts you
Clinical safety officer, or the person who names one	Paid, contracted, with the hazard log	6 months plus

Never put his name or his photograph on anything without written permission. A clinician's name on a product implies endorsement, and it is his registration at risk, not yours.